When the Family Disagrees About Care

A short guide to agreeing on what matters before agreeing on what to do.

Your brother has one answer and your sister has another, and each of them is sure. The decision is close, so every conversation goes straight to what to do.

This guide explains why it helps to talk about something else first: what matters to your loved one, what matters to each of you, and what nobody knows yet. It doesn't pick a side or an option.

Why what matters comes before what to do

An answer is a solution: this arrangement or that one. Underneath each relative's answer is something they are trying to protect, and a hard decision is easier to discuss once those things have been named.

Seeing it differently is ordinary. The Family Caregiver Alliance, writing for brothers and sisters caring for a parent, says that everyone in a family handles a parent's aging differently, and that differences in how each one sees the parent's situation are common. [1]

You also don't need full agreement to go on. The same organization says that not every issue in caregiving and decision-making can be solved, and that sometimes it is important to accept approximations of a good solution. [2]

- Three things to keep in view while the decision is live: your loved one's own voice, an honest record of where the family differs, and the information you still need.

Listen to your loved one first

The National Institute on Aging says the best way to know what someone needs is to ask them directly. AARP's planning guide for families goes further: every family caregiving plan must center on the wishes of the person receiving the care. [3] [4]

Asked on its own, “What's important to you?” doesn't always bring out much detail. A federal toolkit on person-centered thinking, written for counselors, says so, and suggests also asking a person to walk through a typical day, with a good day on one side of the page and a bad day on the other. [5]

Then write down what they say the way they said it. The Institute for Healthcare Improvement, in guidance written for health systems, tells care teams to use the person's own words as much as possible, to give them time to think, and not to ask an overwhelming number of questions. [6]

- Keep what they have said apart from what the family believes they'd want. “She'd never want that” is a belief until your loved one has said it themselves.
- A few of their own sentences, with the date, are enough to start.

Then listen to each other

The Family Caregiver Alliance says a successful family meeting gives everyone a chance to be heard, and suggests saying “I need” rather than “You should.” Asking each relative the same questions, what worries you and what do you hope for, makes that easier to do. [2]

A hope and an offer of help are different things, so ask about them separately. The Family Caregiver Alliance's advice is to ask each person for what they can really give. The National Institute on Aging says no one can be expected to do everything, and to be realistic about how much you can do and what you are willing to do. [1] [7]

- Write each person's worry, hope and offer in three separate places.
- Where two people's expectations pull against each other, write both down. Don't choose between them.

Put what's shared and what isn't on one page

Shared decision criteria are what a family agrees a choice will be measured against, settled before the options are compared. This guide doesn't write yours. That work needs a full picture of your loved one's situation beside it.

What you can do now is record. Put agreement and disagreement on the same page, and name the tradeoffs you can see without settling them. The federal toolkit notes that there are tradeoffs between the things that are important to us, and that some take priority over others at different times in our lives. [5]

Leave what's unsettled where everyone can see it. The Family Caregiver Alliance reminds families that a meeting is not a one-time event, which puts less pressure on everyone to resolve everything in one sitting. When a family is divided on a big issue, it says, it often helps to invite a neutral outside facilitator. [2]

What this means for your family

- The first thing to agree on is what matters. What to do comes after.
- Your loved one's words go at the top of the page, as they said them.
- A disagreement written down plainly is progress. You don't have to make it disappear to move on.
- The questions nobody in the family can answer go on a list, each with the kind of person who can: their doctor or nurse, a social worker, or a care consultant.

What this guide doesn't do

It doesn't say who is right, settle anything between you, or tell you which option to choose. The advice on asking, listening and meeting comes from the sources below. The order, and the practice of keeping your loved one's wishes separate from the family's, are the Family Care Roadmap's own.

Ready for the next step?

You can see why what matters comes first. The care interview adds the other half: it takes you through your loved one's situation one area at a time, so the family's priorities sit beside what the decision is actually about, and turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Family Caregiver Alliance, Caregiving with Your Siblings
2. Family Caregiver Alliance, Holding a Family Meeting
3. National Institute on Aging, Does an Older Adult in Your Life Need Help?
4. AARP, Prepare to Care: A Planning Guide for Families
5. Administration for Community Living (National Center on Advancing Person-Centered Practices and Systems), Office of Healthcare Information and Counseling Person-Centered Thinking Toolkit
6. Institute for Healthcare Improvement (Age-Friendly Health Systems, with The John A. Hartford Foundation), “What Matters” to Older Adults? A Toolkit for Health Systems to Design Better Care with Older Adults
7. National Institute on Aging, Sharing Caregiving Responsibilities